



Original article

Suicide Among Young People in the Americas

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A B S T R A C T

Purpose: To examine suicide mortality trends among young people (10–24 years of age¹) in selected countries and territories of the Americas.

Methods: An ecological study was conducted using a time series of suicide mortality data from 19 countries and one territory in the Region of the Americas from 2001 to 2008, comprising 90.3% of the regional population. The analyses included age-adjusted suicide mortality rates, average annual variation in suicide mortality rates, and relative risks for suicide, by age and sex.

Results: The mean suicide rate for the selected study period and countries/territory was 5.7/100,000 young people (10–24 years), with suicide rates higher among males (7.7/100,000) than females (2.4/100,000). Countries with the highest total suicide mortality rates among young people (10–24 years) were Guyana, Suriname, Nicaragua, El Salvador, Chile, and Ecuador; countries with the lowest total suicide mortality rates included Mexico, Venezuela, Cuba, and Brazil, and the U.S. territory of Puerto Rico. During this period, there was a significant increase in suicide mortality rates among young people in the following countries: Argentina, Chile, Ecuador, Mexico, and Suriname; countries with significant decreases in suicide mortality rates included Canada, Colombia, Cuba, El Salvador, and Venezuela. The three leading suicide methods in the Americas were hanging, firearms, and poisoning.

Conclusions: Some countries of the Americas have experienced a rise in adolescent and youth suicide during the study period, with males at a higher risk of committing suicide than females. Adolescent and youth suicide policies and programs are recommended, to curb this problem. Methodological limitations are discussed.

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IMPLICATIONS AND
CONTRIBUTION

Suicide mortality among young people is a growing public health issue in the Americas. To develop effective suicide prevention programs and policies, public health policy and decision makers should consider the magnitude of the problem in different settings, groups most at risk, and the risk of suicide mortality over time.

¹ The World Health Organization defines young people as individuals between the ages of 10 and 24 years. Adolescents comprise the 10- to 19-year age group, and youth the 15- to 24-year age group.

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A decade into the 21st century, the adolescent and youth population is the largest cohort in the history of the Americas, representing 26% of the total population [1]. These young people face a myriad of obstacles excluding them from health that are closely linked with poverty, marginalization, and discrimination. Whereas morbidity and mortality are relatively low during